

Africa CDC African Epidemic Services (AES) Epidemiology Fellowship 2026–2028

Fellowship Nomination — Surveillance Science for Community Health

FairBanks Community Health Intelligence Platform (FCHIP)

Professional development that returns outbreak intelligence to FCHIP and Uganda's communities

Your health, our mission.



Health for All — Obulamu eri Bonna · Afya kwa Wote · Oburamu bwa Boona

3 months training in Addis Ababa · 21 months field placement · fully funded

1. Fellowship purpose

Africa CDC's AES fellowship builds outbreak investigation, surveillance, and data-analysis skills over two years — including a Public Health Informatics track. FairBanks needs this depth to make FCHIP's surveillance claims scientifically sound and operationally useful in real communities.

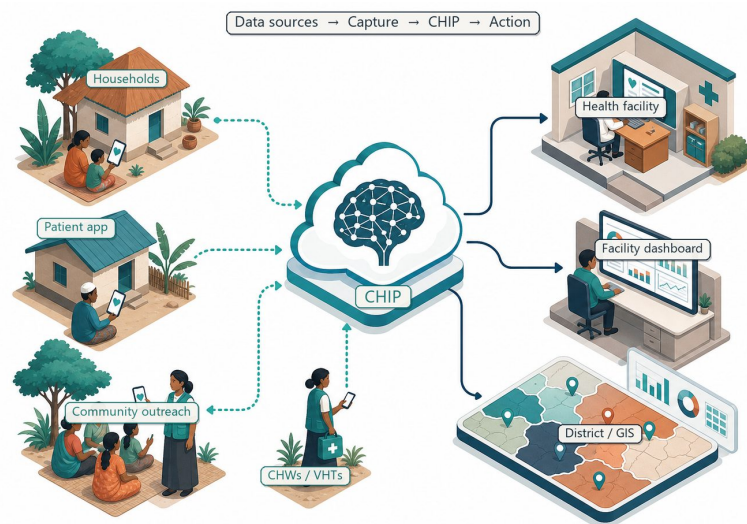
We nominate an eligible FairBanks public-health professional (African Union citizen, under 35, employed in public health with degree and three years' experience) for the Epidemiology or Informatics track. Africa CDC gains a fellow already embedded in community health work; FairBanks gains skills that return directly to CHWs, clinics, and partner districts.

2. Why surveillance science needs community roots

Predictive community platforms fail without trained people who understand surveillance science — not only software. Outbreak signals from Community Health Workers and Village Health Teams only become action when someone can investigate cases, analyse trends, and design informatics pipelines with discipline.

Africa needs more young epidemiologists and informaticians grounded in primary care realities. FairBanks sits inside that reality: live outreach, maternal and child health, chronic screening, and a growing intelligence layer on the community health cascade.

3. Skills that return to FCHIP



FCHIP on the community health cascade

A FairBanks fellow will complete AES training and bring methods home: standardised community signal → investigation → response workflows; surveillance analysis for fever clusters, maternal risk, and medicine stock-outs; and informatics design that improves CHW data quality.

Employer support ensures learning returns to Community Reach programmes — not a certificate on a shelf. Supervisors and CHW leads will receive cascade training so Africa CDC's investment multiplies across Kampala catchments and partner sites.

4. Priority application areas

- Outbreak workflows: community fever reports trigger structured investigation and pharmacy pre-stocking before surges spread.
- Informatics: validation rules and data-quality pipelines for offline CHW capture feeding FCHIP.
- Training cascade: teach CHW supervisors better data use and early-warning interpretation.

5. Employer commitment

FairBanks Medical Centre and FairBanks Community Reach will release the fellow for Addis Ababa training, provide a structured field placement linked to outreach and facility data flows, and assign internal mentors for surveillance quality improvement.

We cover counterpart supervision time, device access for field work, and integration of fellowship outputs into FCHIP modules at no extra cost to Africa CDC beyond the fellowship package.

6. Two-year fellowship pathway

Phase	Activities
Months 1–3	Core epidemiology and informatics training in Addis Ababa
Months 4–12	Field placement — outbreak workflows and CHW data quality at FairBanks sites
Months 13–24	Advanced surveillance modules; district partner engagement; FCHIP alert validation
Throughout	Monthly learning reports; Africa CDC network participation; community feedback loops

7. Fit to AES fellowship criteria

Criterion	How FairBanks responds
AU citizen under 35	Ugandan staff candidate within eligibility range
Employed in public health	FairBanks clinical / community health employment
Degree + 3 years' experience	Nominee will meet stated requirements
Epidemiology / Informatics tracks	Direct fit to FCHIP surveillance and data-quality goals

8. Mutual value

FairBanks / FCHIP gains: World-class epidemiology training; stronger surveillance modules; stipend, travel, insurance, hardware, and software covered; continental AES network access.

Africa CDC gains: Fellow employed in live community health work; field placement linked to real CHW and facility data; Uganda contribution to Africa's epidemic readiness; informatics track aligned with digital community health tools.

9. Selection ask

We ask for selection into the 2026–2028 AES Epidemiology or Public Health Informatics cohort.



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